

CANCER CARE ASSOCIATES Hematology Division

March 28, 2024

CLINICAL NOTE

Patient: James Anderson

MRN: AML016

DOB: 05/03/1975

Date of Service: 03/28/2024

Mr. Anderson is a 48-year-old gentleman with favorable-risk AML (NPM1+/FLT3-) diagnosed July 2023, currently day +8 of cycle 4 consolidation with HiDAC.

HPI: Diagnosed 07/21/23 after presenting with gingival hypertrophy and WBC 89K. BM showed 68% blasts, normal cytogenetics, NPM1 mutation (type A) without FLT3. Completed 7+3 induction 07/28/23 with CR achieved day 29. Now on cycle 4 planned HiDAC consolidation.

Today reports mild fatigue but working from home 20 hrs/week as software engineer. No fever, bleeding, or infections. Tolerating consolidation well.

EXAM:

Afebrile, BP 118/72, HR 72

Well-appearing, no acute distress

No gingival hypertrophy

Clear lungs, no murmurs

No hepatosplenomegaly

No petechiae/bruising

LABS TODAY:

WBC 3.2 (ANC 1850)

Hgb 11.8

Plt 142K

Cr 0.8, LFTs normal

ASSESSMENT/PLAN:

- Favorable risk NPM1+ AML in ongoing CR after induction
- Excellent tolerance, maintaining good QOL
- MRD assessment by NPM1 RT-PCR after cycle 4
- If MRD negative, transition to observation

Jennifer Liu, NP